



# Health Trail

A concept for a family caregiving app,  
shared early for honest critique.

# The idea

**What this is.** A phone app for the family member who becomes the point person for someone else's care: a parent in a nursing home, a spouse in rehab, a child in ongoing treatment. Not for the patient, and not for clinicians. For the son or daughter making the calls.

**Where it comes from.** Lived experience: years of a parent's care. The falls, the facilities, the transfers, the therapies, the bills that appeared from providers no one chose, the records that had to be fought for. Every screen in this document exists because a family needed it at some point and did not have it.

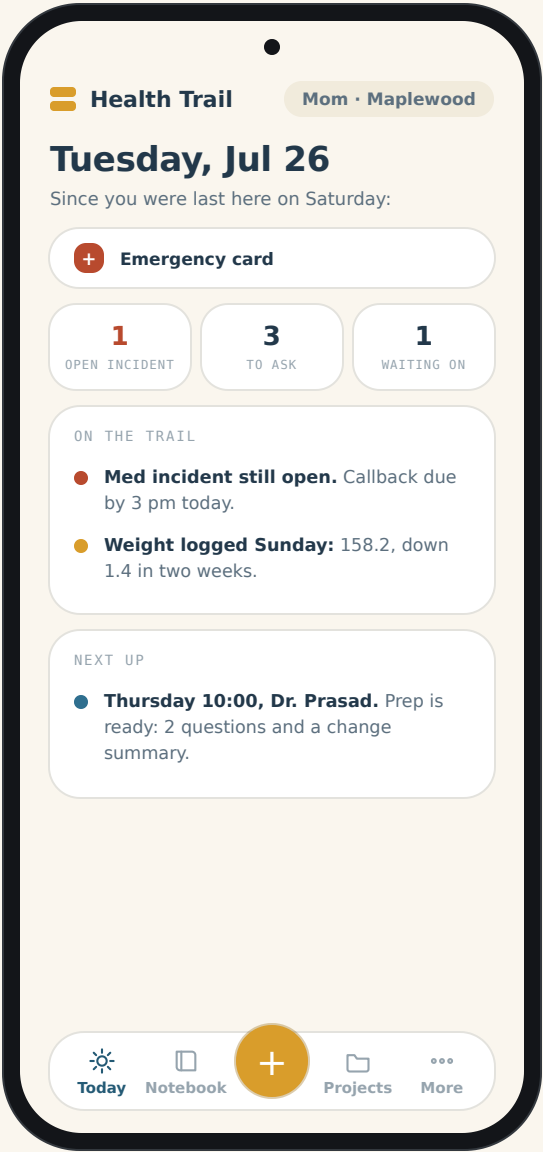
**The core idea.** In practice, the family caregiver is the information hub, because providers rarely talk to each other. This app is that hub given structure: one trail of every call, visit, incident, medication change, document, and dollar, kept for years and searchable in seconds.

# The ground rules

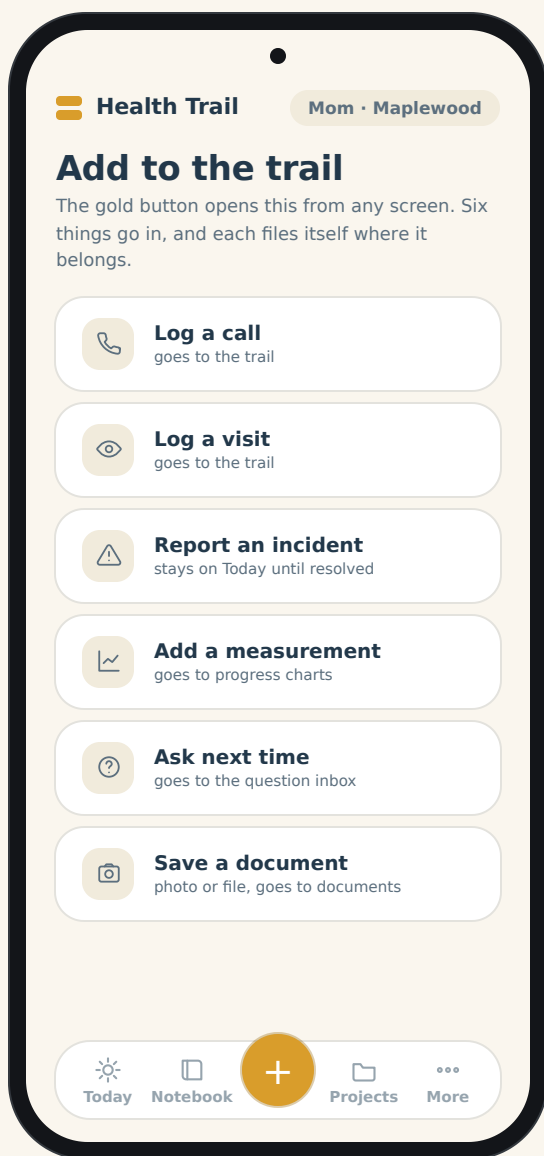
- One hundred percent free and completely private. No account, no cloud, no company scanning the data. Nothing ever leaves the phone.
- The information belongs entirely to the person who entered it, and exports in full at any time. Free and open source, built and carried by one person.
- It records and organizes. It never diagnoses, never reminds about doses, never advises, and never interprets. Where it counts things, it says so and counts only.
- Sharing happens by generating a PDF or text on the phone and sending it through whatever app the person already uses.

The 27 screens that follow are concept mockups. Every person, facility, medication, and event in them is fictional, composed to illustrate. Feedback questions are on the last page.

**Getting in.** The first minutes decide everything. Most people open an app like this in a hallway or a waiting room, not on a quiet Sunday.

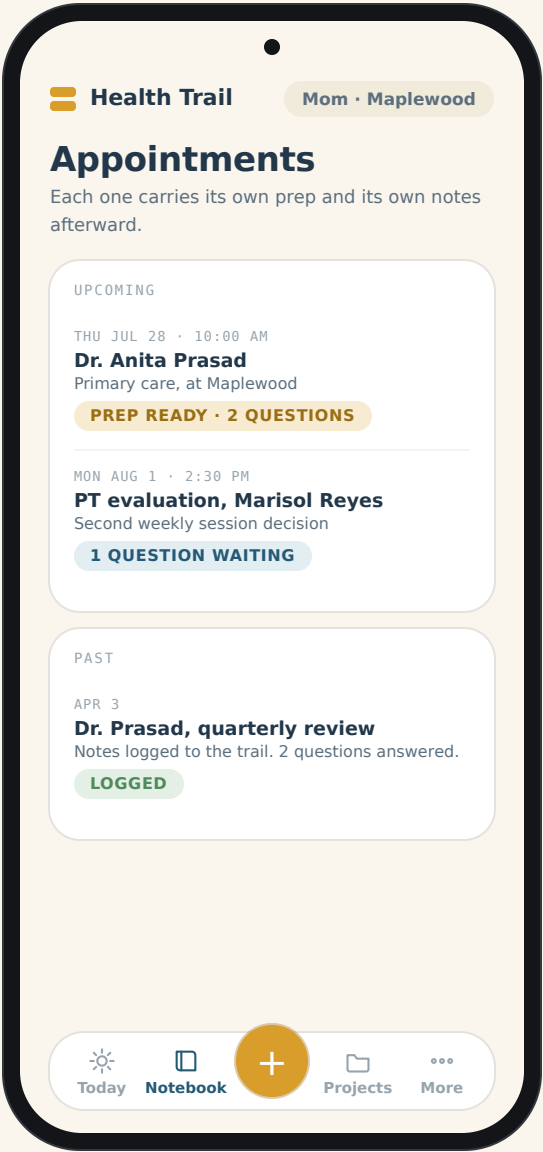


Setup is built for a family mid-crisis. Three fields and you are in. The full setup can wait for a calmer day.



## 02 • THE CAPTURE BUTTON, OPEN

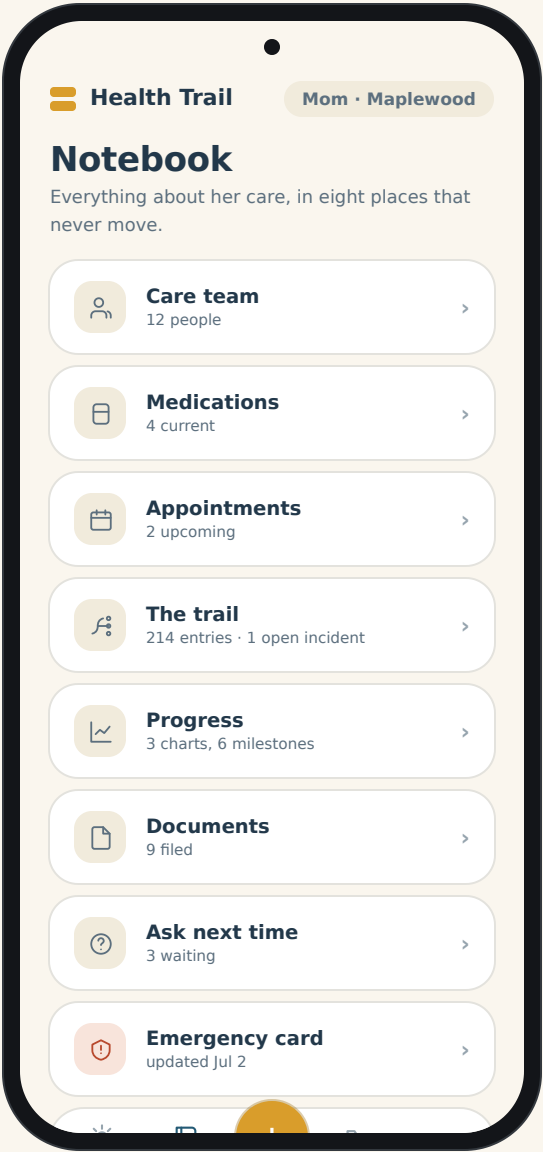
An empty app that teaches itself. The first suggestion is the emergency card, because that is the two minutes that matter most later.



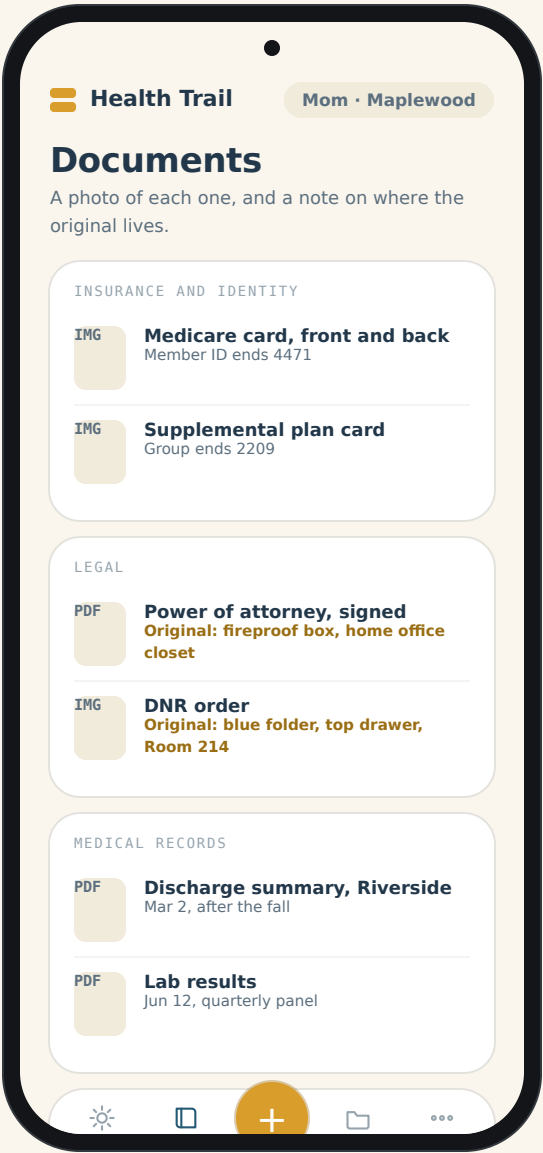
03 • APPOINTMENTS, NOW A REAL SECTION

Logging a phone call takes about fifteen seconds: who, one line, save. Capture speed decides whether a record like this survives real life.

**Where everything lives.** Four tabs along the bottom of every screen, and one gold button in the middle. The button is the only way anything goes in. The tabs are where it lives afterward.



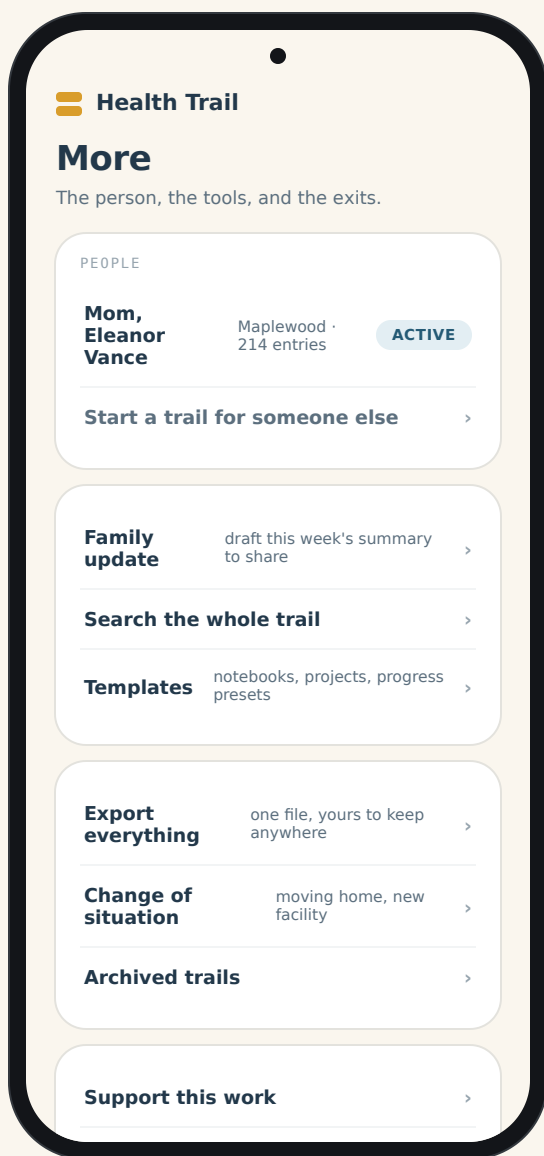
The Notebook tab is the digital version of the binder many families keep: eight fixed sections with live counts. Nothing ever moves.



05 • DOCUMENTS, THE MISSING BINDER TAB

Documents get photographed and filed, with a note on where the physical original lives. The DNR entry saying which drawer it is in has saved real families real hours.

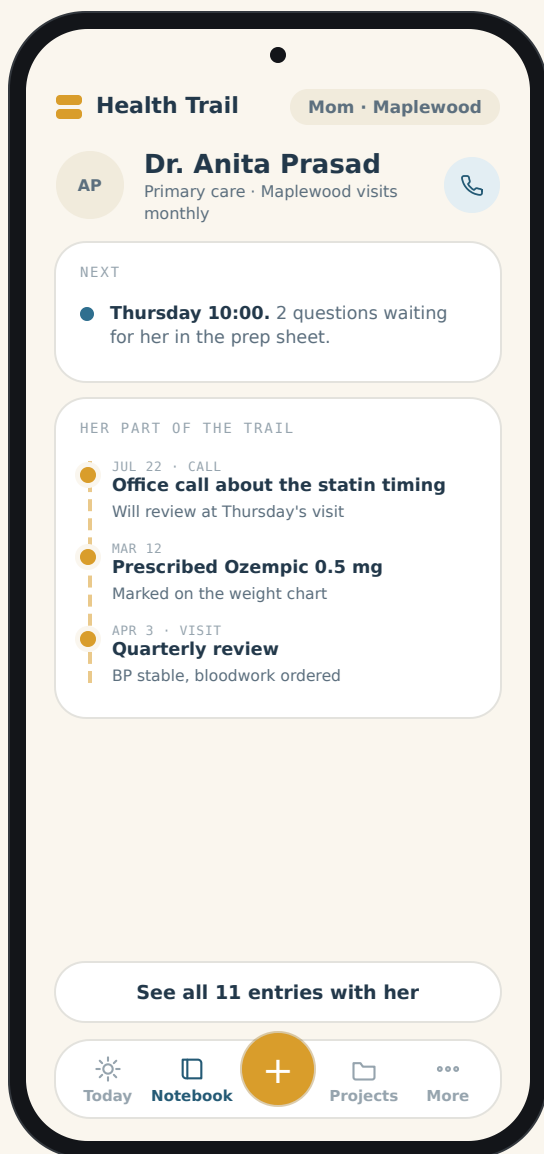




## 06 · MORE, TOOLS AND EXITS

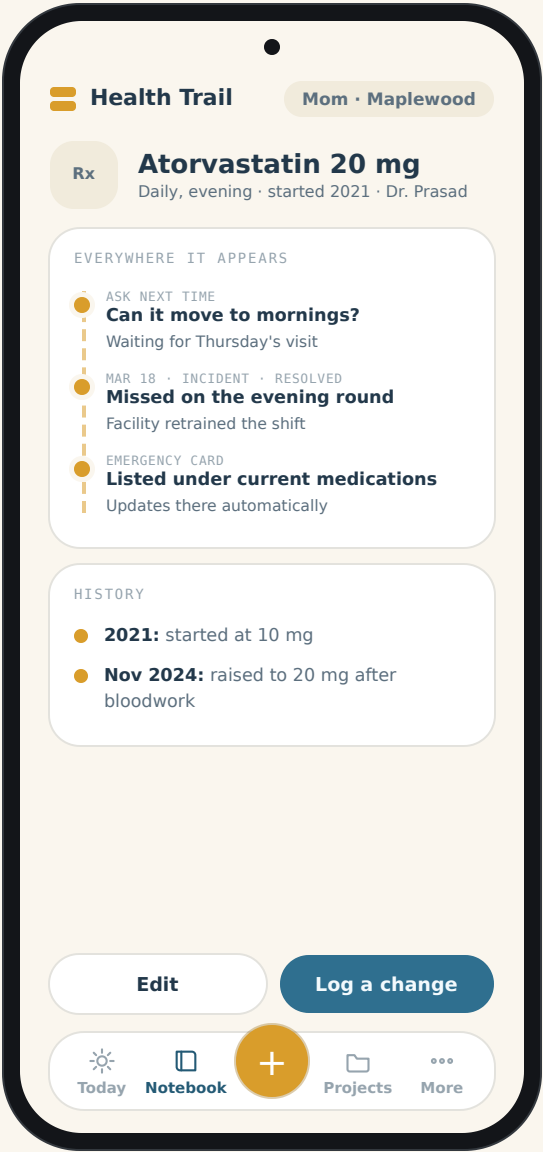
The last tab holds the person switcher, tools, and exits, including a full export. There is no account anywhere in this app, so there is nothing to cancel and nothing held hostage.

**Everything connects.** The rule underneath the whole app: every item links to everything it touches. People, medications, incidents, appointments, and documents all know about each other.



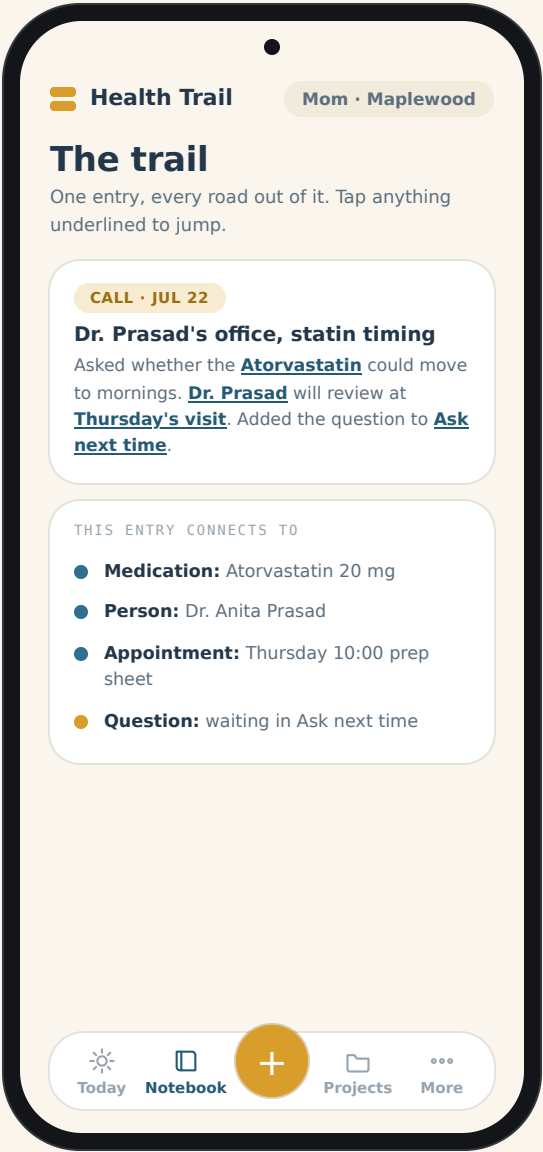
#### 07 · A PERSON PAGE PULLS ITS WHOLE STORY

Every person's page assembles their entire history automatically: every call, visit, and prescription involving them, plus the questions waiting for them.



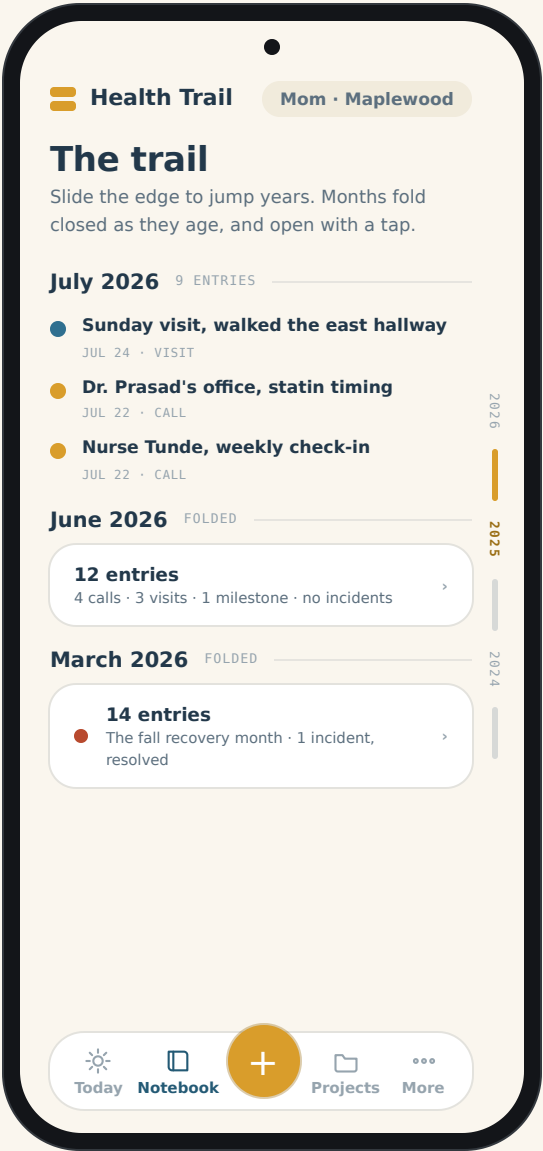
08 · A MEDICATION KNOWS ITS OWN STORY

A medication knows its own story: the pending question about it, the incident where it was missed, its dose history, and its place on the emergency card.



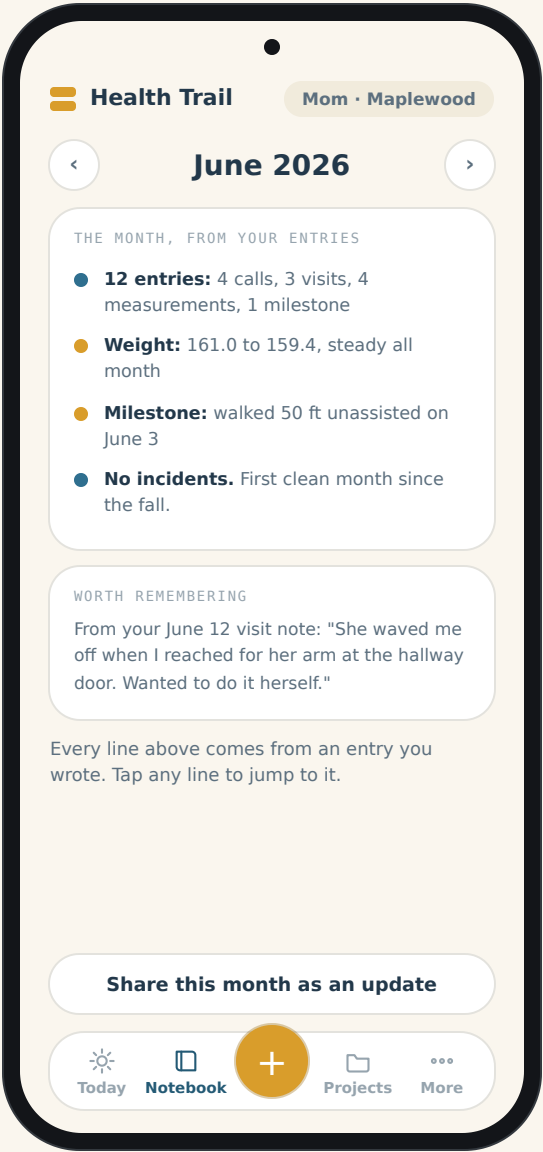
Any single entry shows every road out of it. The rule is no dead ends: you never need to remember where something was filed.

**Zooming out.** A record like this can span years. It has to stay readable at that scale.

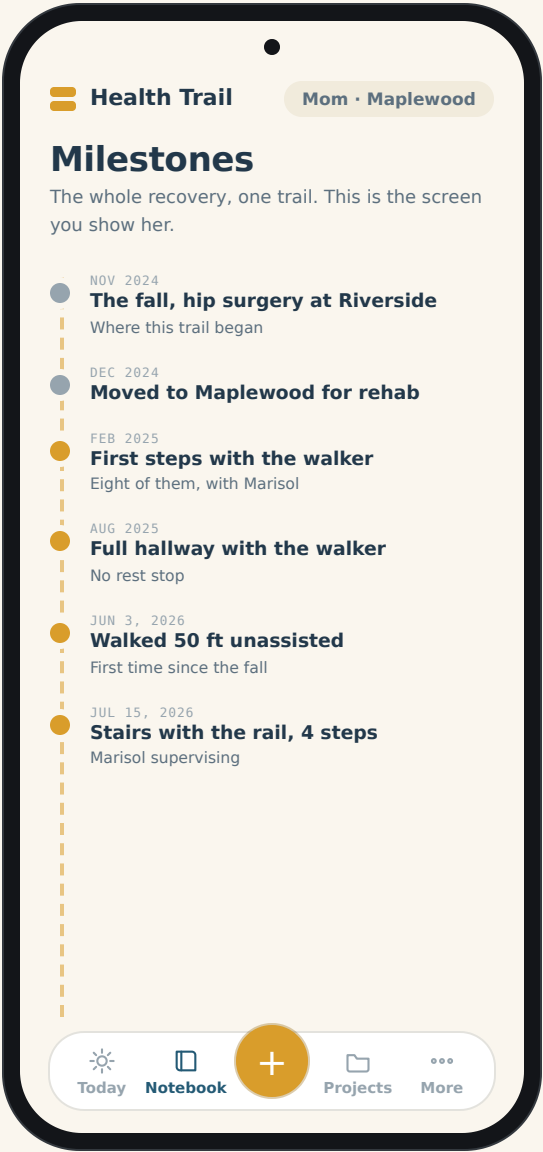


10 · YEARS OF TRAIL, STILL LIGHT

Years of history stay light. Recent months show full entries, older months fold into one line summaries, and the right edge jumps between years.



Each month can be read back as a summary built line by line from real entries, never generated or guessed. Tap any line to jump to its source.



All milestones on one path: the whole recovery from the fall to the stairs. Families say this is the screen they would show the person themselves.

**Zooming in.** And it has to survive scrutiny at the smallest scale: one incident, one day.

 **Health Trail**

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# Missed evening meds

RESOLVED IN 9 DAYS

THE THREAD, START TO FINISH

MAR 18

**Reported**

Lisinopril and Atorvastatin missed on the evening round. Spotted on the chart during a visit.

MAR 19

**Called the front desk**

Promised a callback from the Director of Nursing

MAR 21

**No callback. Called again**

Spoke to T. Okonkwo directly, review promised

MAR 24

**Escalated to the administrator**

Meeting set for the 26th

MAR 26

**Facility response**

Evening shift retrained, double-check added to the round

MAR 27

**Marked resolved**

No repeat since

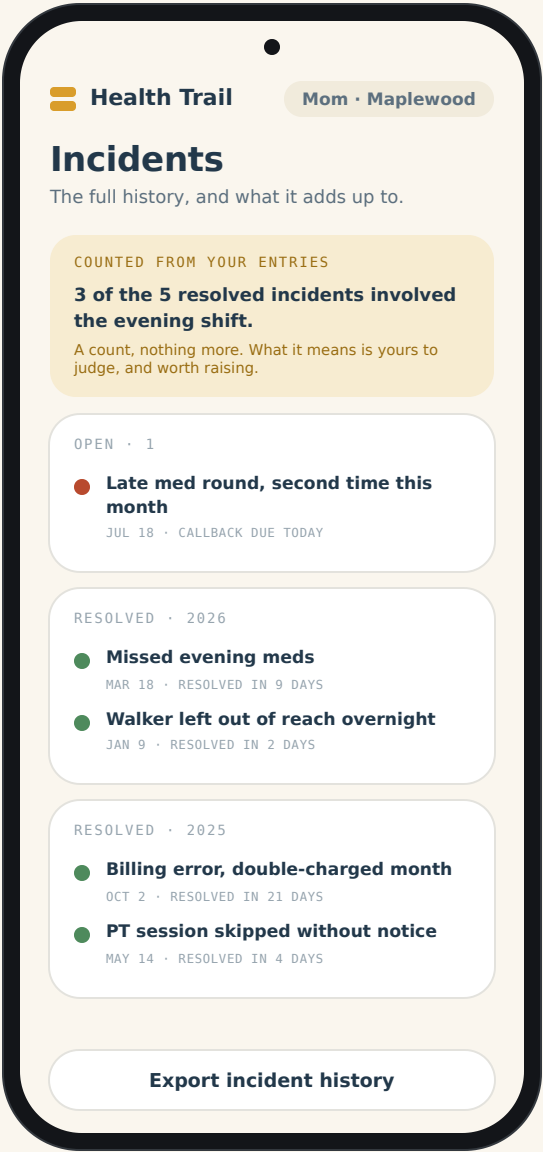
If it ever happens again, this thread is the record you bring to the meeting.

Export this thread as PDF

13 • ONE INCIDENT, FOLLOWED THROUGH

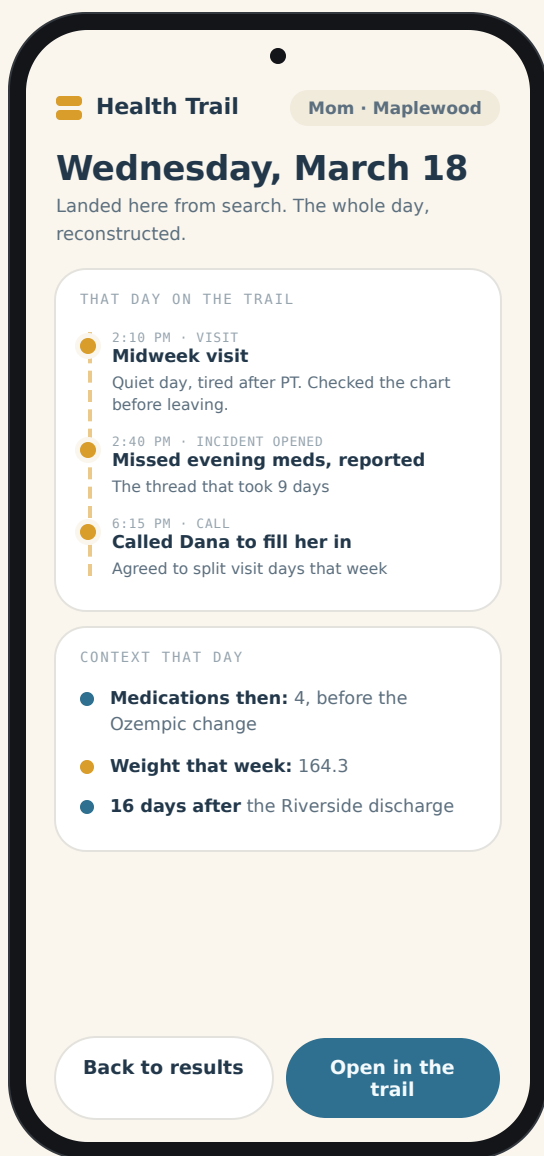
One incident followed start to finish: reported, calls made, escalated, the facility's response, resolved in nine days. This thread becomes the record if it ever happens again.





14 · THE HISTORY, AND THE COUNT

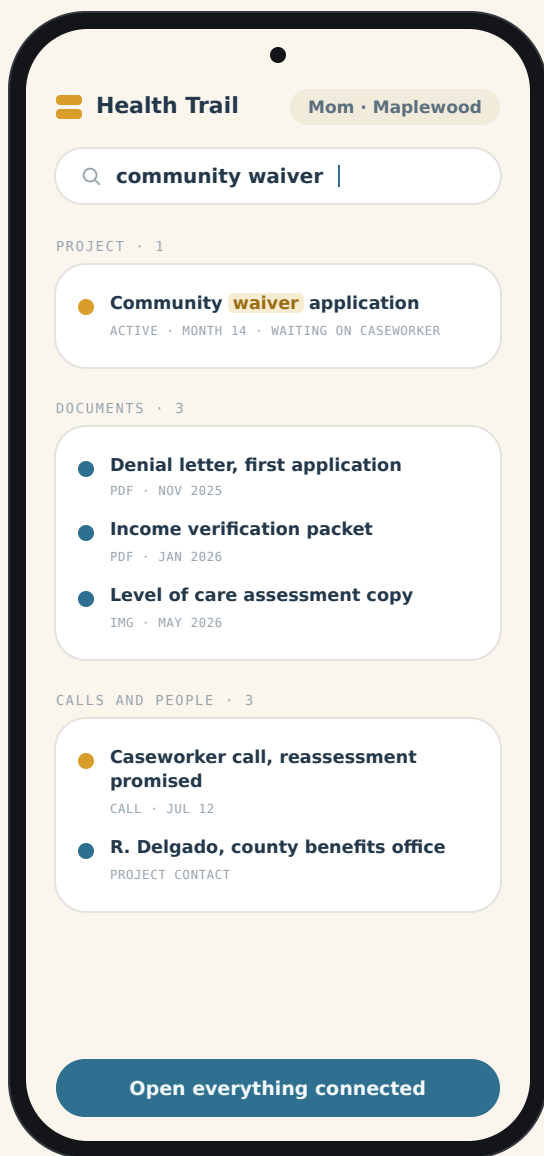
The incident history across years, with a plain count on top: three of five involved the evening shift. The app counts. It never interprets.



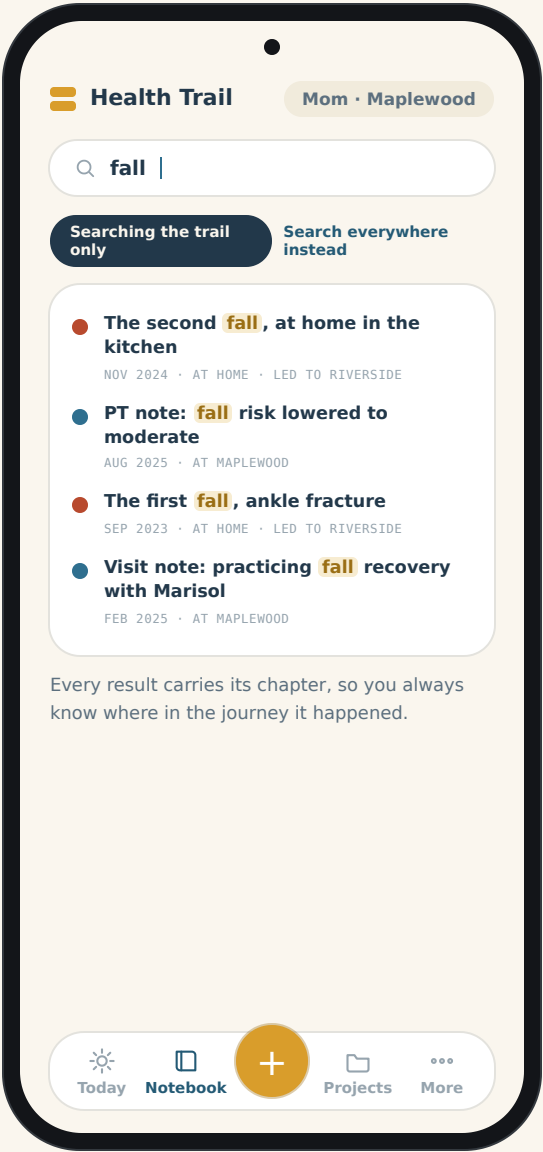
## 15 • ANY DAY, RECONSTRUCTED

Any date can be reconstructed: everything logged that day, plus context like the medication list and weight at the time.

**Search, everywhere.** One universal bar on the home screen, a scoped bar inside every section, and results that pull associated information together.

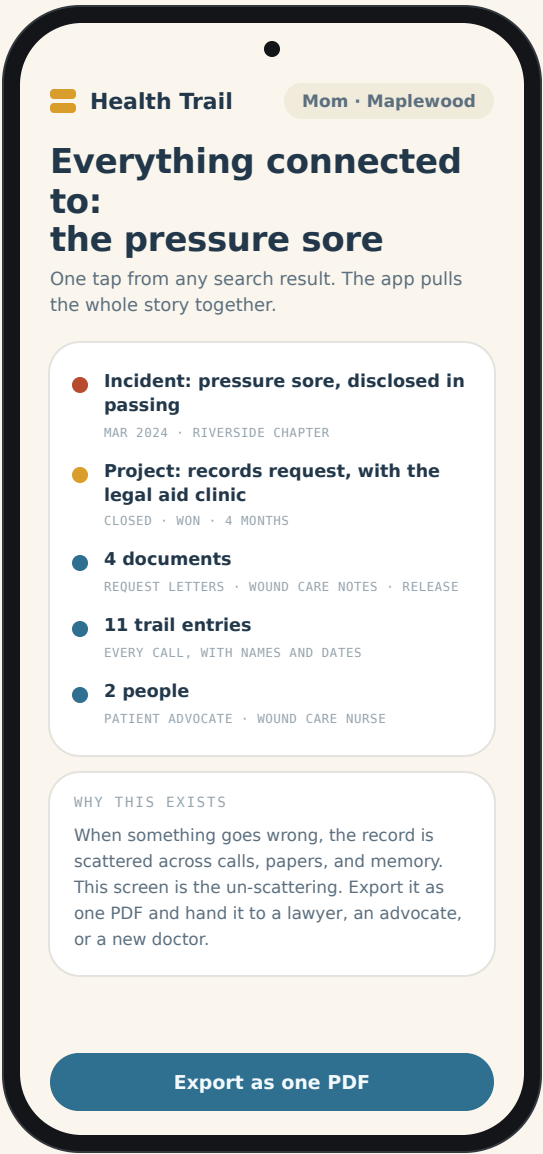


One search bar on the home screen reaches everything. A query like a benefits program returns the project, its documents, the calls, and the caseworker, grouped.



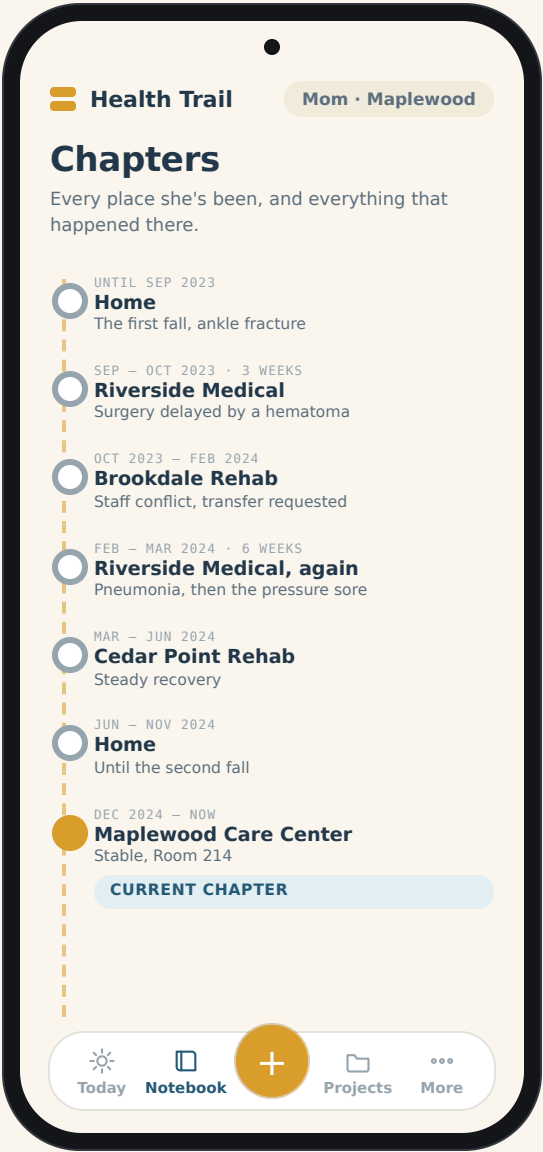
17 · SCOPED SEARCH, INSIDE A SECTION

Inside any section, search is scoped to just that section, with one tap to widen. Every result carries its place in the journey.

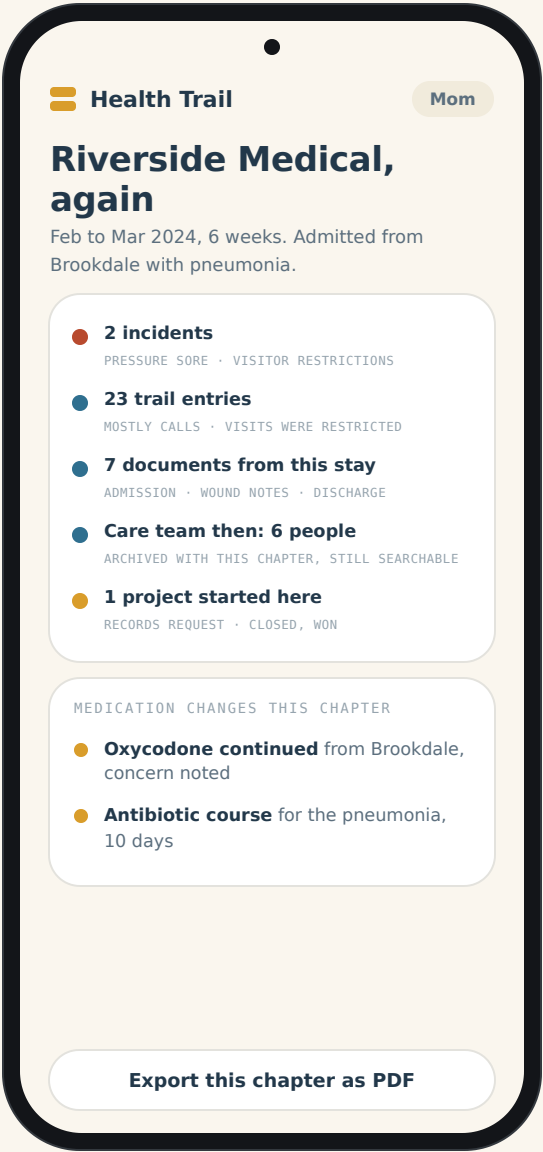


From any result, one tap assembles everything connected to it: the incident, the legal project, the documents, eleven dated calls, the people. Exportable as one PDF for a lawyer, advocate, or new physician.

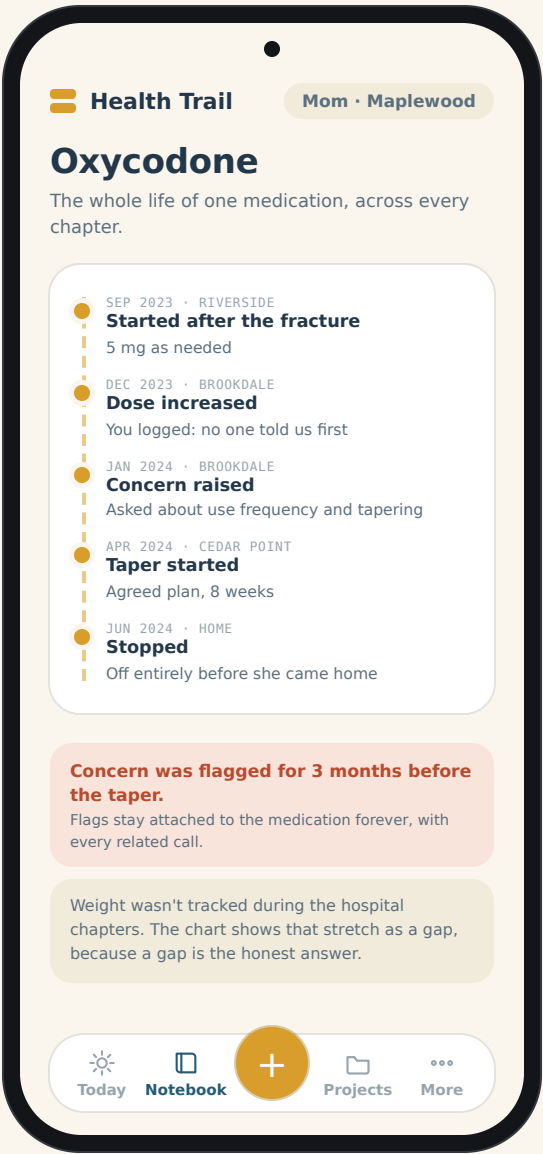
**The journey.** Real care moves: home, hospital, rehab, back again.  
The app models that directly.



Care rarely happens in one place. Chapters make the journey itself a first class structure: home, hospital, rehab, hospital again, rehab, home, and the current facility.



Inside one chapter: why she was admitted, what happened there, the documents from that stay, and the care team of that period, archived with it but still searchable.

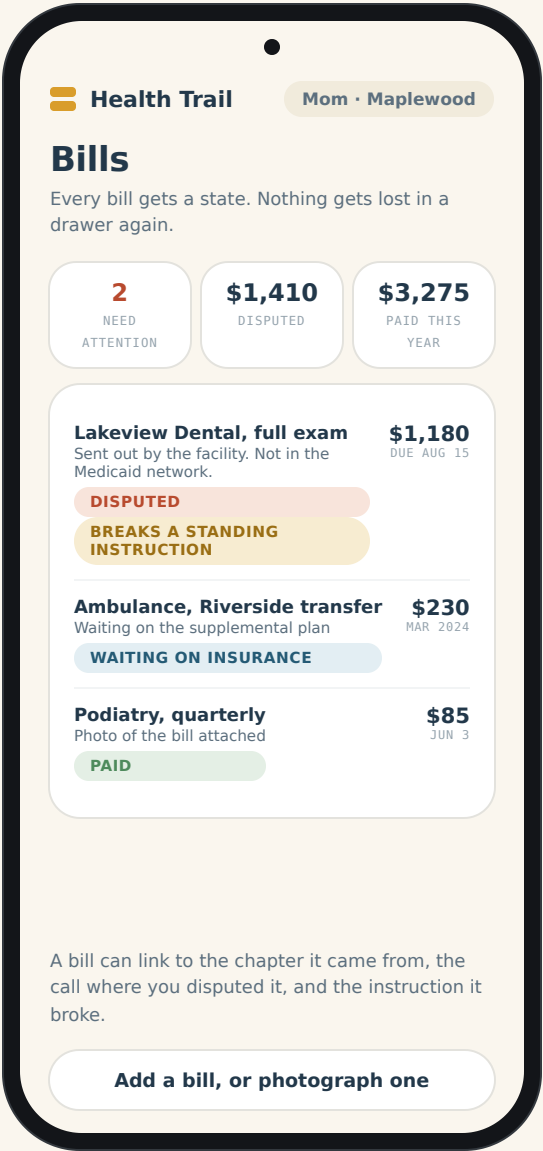


21 · ONE MEDICATION, EVERY CHAPTER

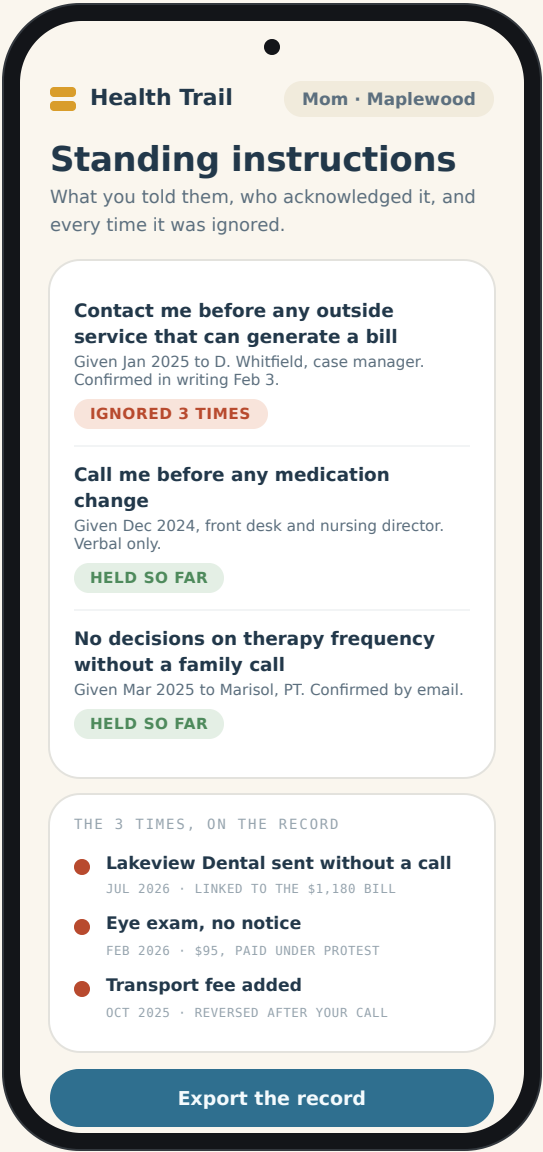
A medication followed across every chapter, including a concern flag that stayed attached for three months before the taper. Where data was never captured, charts show a gap rather than a guess.



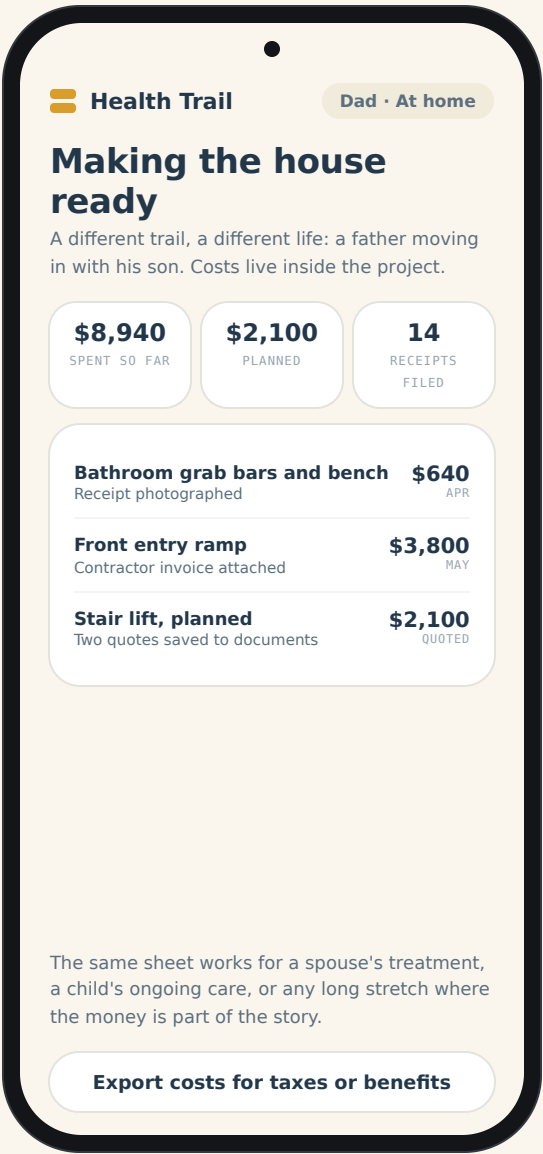
**Money and the rules.** Surprise bills, running costs, and the instructions families give facilities, together with what happened to those instructions.



Bills get states: disputed, waiting on insurance, paid. The surprise out of network referral is a real and common failure, and here it links to the instruction it broke.

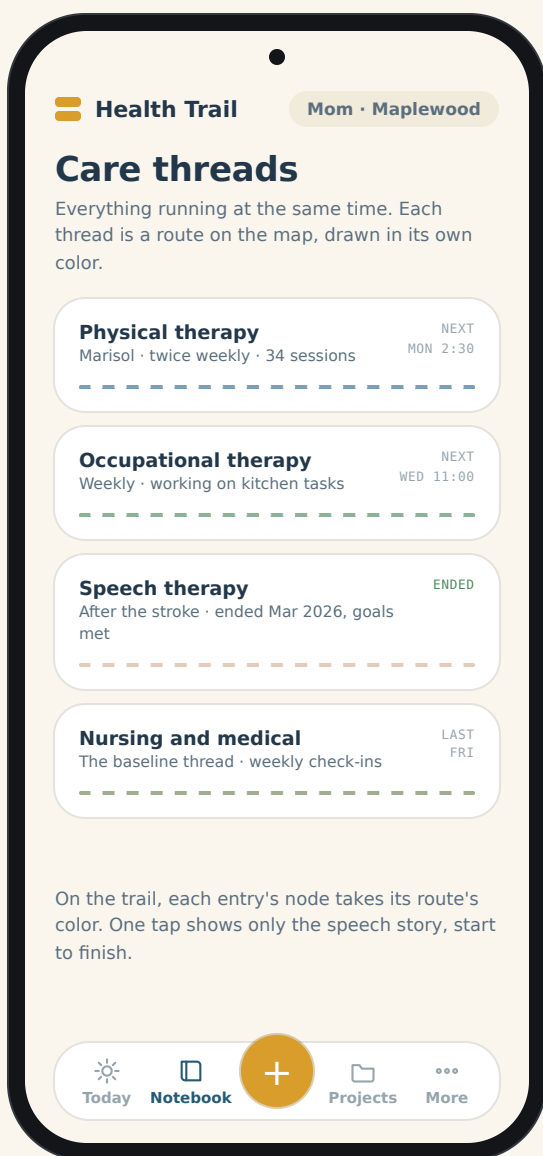


Standing instructions record what the family told the facility, who acknowledged it, and every documented time it was ignored. It turns a memory into a record.



The same cost sheet serves a different life entirely: a father moving in with his son, and the expense of making the house ready. The app was never only about seniors.

**Many things at once.** Several therapies can run in parallel. The interface keeps them straight without making entry harder.



## 25 · PARALLEL THREADS, ONE TRAIL

Multiple therapies running at once each become a thread, drawn like a route on a trail map. All of them land on the same single timeline, and the timeline can be filtered to any one story.

 **Health Trail**

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## New entry

Answer what you can. Skip the rest. A half note beats no note.

**When was this?** roughly is fine  
Today Yesterday This week Not sure

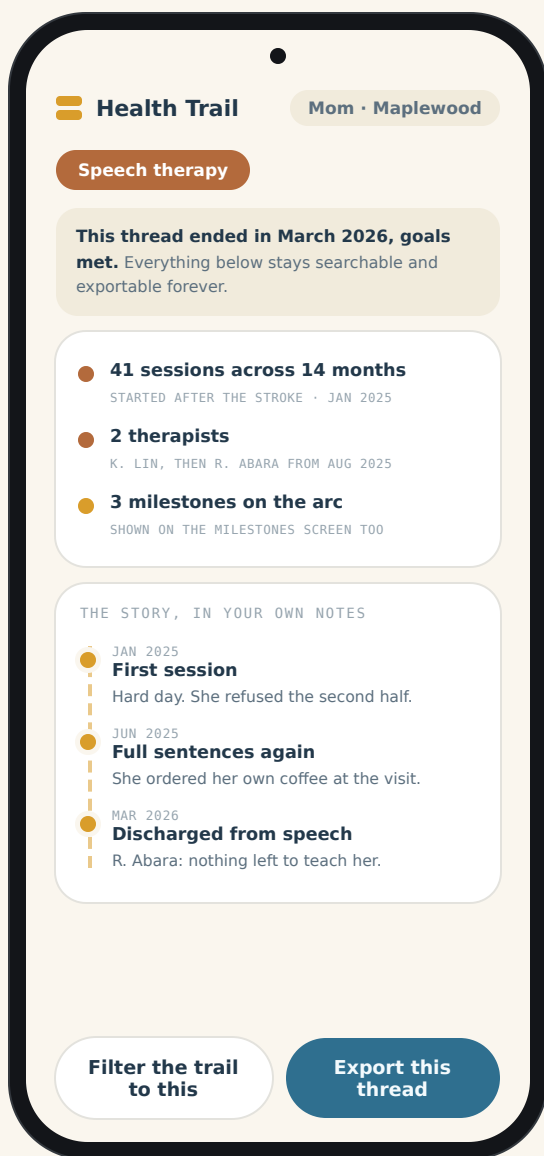
**What is it part of?** skip if you don't know  
☒ PT ☐ OT ☐ Speech Not sure yet

**What happened?**  
Therapist mentioned swallowing exercises twice a day, and a follow up in two weeks. I think with the same person.

**Save what you have**

"Not sure yet" goes to the Unfiled tray. The app suggests a home by matching words, and you confirm with one tap. It never files anything without you.

Capture assumes imperfect memory. Every question is optional, dates can be rough, and half remembered notes go to a tray where the app suggests a home and the person confirms. It never files anything on its own.



## 27 • ONE THREAD, WHOLE STORY

A finished thread keeps its whole story: forty one sessions, two therapists, the milestones, and the family's own notes, from the hard first day to the discharge.

# Open questions

Honest and specific beats kind. These are the questions where a clinical eye matters most:

- **The emergency card.** Are those the fields a first responder or covering physician actually needs? What is missing, and is anything on it unwise?
- **Medications are a record only.** No reminders, no alerts, deliberately. Right call for this audience, or a mistake?
- **The appointment prep sheet.** From the other side of the desk: does a family arriving with questions and a change summary help a fifteen minute visit or derail it?
- **Incidents and standing instructions.** A fair, constructive record, or adversarial in a way that would poison the relationship with a facility?
- **What is clinically naive here?** Where would this break against how care actually works?
- **What is missing** that you have watched families need and not have?